



HEXAMED

DIAGNOSTICS & SPECIALTY CLINICS

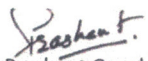
Your Right to Right Healthcare...

Laboratory | Radiology | Polyclinic | Pharmacy

Barcode No	89781196	Lab No	16952502020007
Patient Name	Mr.ROYAL DEEP	Reg Date	02/Feb/2025 09:09PM
Age/Sex	7 YRS/Male	Sample Coll. Date	02/Feb/2025 08:14 PM
Referred By	DR. NEERUPAM GUPTA	Sample Rec.Date	03/Feb/2025 08:30 AM
		Report Date	04/Feb/2025 12:17PM

IMMUNOLOGY

Test Name With Methodology	Result	Unit	Biological Ref.Interval
Lead (Blood)			
Lead ICP-MS	24.55	ug/L	< 150


Dr. Prashant Goyal (DCP)
(Director & Chief Pathologist)
Reg. No. DMC-53016

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Barcode No	89781195	Lab No	16952502020006
Patient Name	Mr.ROYAL DEEP	Reg Date	02/Feb/2025 09:08PM
Age/Sex	7 YRS/Male	Sample Coll. Date	02/Feb/2025 08:14 PM
Referred By	DR. NEERUPAM GUPTA	Sample Rec.Date	03/Feb/2025 08:31 AM
		Report Date	03/Feb/2025 02:09PM

IMMUNOASSAY

Test Name With Methodology	Result	Unit	Biological Ref.Interval
TTG- IgA (Tissue Transglutaminase IgA Antibody)			
Tissue Transglutaminase Ab IgA	3.89	U/mL	<4.0 Negative 4-10 Weak Positive >10 Positive
Serum, ELISA			

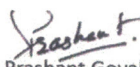
Comments:

Useful for Evaluating patients suspected of having celiac disease, as Screening test for dermatitis herpetiformis, in conjunction with endomysial antibody test & Monitoring adherence to gluten-free diet in patients with dermatitis herpetiformis and celiac disease. A definitive diagnosis of celiac disease requires a jejunal biopsy demonstrating villous atrophy. Those individuals with positive laboratory results should be referred for small intestinal biopsy to confirm the diagnosis. If patients strictly adhere to a gluten-free diet, the unit value of IgA-anti-tTG should begin to decrease within 6 to 12 months of onset of dietary therapy.

CAUTIONS

This test should not be solely relied upon to establish a diagnosis of celiac disease. It should be used to identify patients who have an increased probability of having celiac disease and in whom a small intestinal biopsy is recommended. Affected individuals who have been on a gluten-free diet prior to testing may have a negative result. For individuals who test negative, IgA deficiency should be considered. If total IgA is normal and tissue transglutaminase (tTG)-IgA is negative, there is a low probability of the patient having celiac disease and a biopsy may not be necessary.

If serology is negative or there is substantial clinical doubt remaining, then further investigation should be performed with endoscopy and bowel biopsy. This is especially important in patients with frank malabsorptive symptoms since many syndromes can mimic celiac disease. For the patient with frank malabsorptive symptoms, bowel biopsy should be performed regardless of serologic test results.


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